

Multi-cancer early detection test report

CANCELED

Participant

Participant ID: PID1234
DOB: 09-DEC-1970
Bio Sex: Female

Sample

GRAIL ID: PFT26WZ6E7
Report Date: 26-FEB-2024 / 21:09 PT
Collection Date: 26-FEB-2024 / 12:54 PT

Ordering Provider

Name: ALEXIS A HENGSTEBECK
Location: LOC10GAL1
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Results

No Result Available

Comments:

Test cancelled at the request of the patient.

GRAIL

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Method

GRAIL's Galleri test is a qualitative, next-generation sequencing (NGS)-based screening test for the detection of DNA methylation signals using cell-free DNA isolated from peripheral whole blood. GRAIL's Galleri test is a screening test used to complement existing single cancer screening tests and cannot be used to confirm a cancer diagnosis. The Galleri test is recommended for use in adults with an elevated risk of cancer such as those age 50 years and older. The test has been validated in the Circulating Cell-free Genome Atlas (CCGA) study and in the PATHFINDER study.¹ A test result of "Cancer Signal Detected" may indicate the presence of cancer. The predicted Cancer Signal Origin(s) can inform diagnostic evaluation recommended by a qualified healthcare professional in accordance with professional guidelines. The Galleri test is a screening test, not a diagnostic test.

When a cancer signal is detected, one or two Cancer Signal Origins are reported. If the top Cancer Signal Origin score is equal to or greater than 9.0, then only one Cancer Signal Origin is reported; if the top Cancer Signal Origin score is less than 9.0, the two top Cancer Signal Origins are reported. The Galleri test has 21 possible Cancer Signal Origins: Anus; Bladder, Urothelial Tract; Bone and Soft Tissue; Breast; Cervix; Colon, Rectum; Head and Neck; Kidney; Liver, Bile Duct; Lung; Lymphoid Lineage; Melanocytic Lineage; Myeloid Lineage; Neuroendocrine Cells of Lung or other Organs; Ovary; Pancreas, Gallbladder; Plasma Cell Lineage; Prostate; Stomach, Esophagus; Thyroid Gland; and Uterus.

The Limit of Detection (LOD95) of Galleri test using abnormal coverage is 0.2.

¹CCGA study (NCT02889978) and PATHFINDER study (NCT04241796); please see data at www.galleri.com/test-report.

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Disclaimers

Galleri test performance may be subject to the collection, storage, and transportation of the blood samples. The test is not intended for other sample types. Any sample handling outside of the suggested procedures may affect test performance.

Use of Galleri is not recommended in individuals who are pregnant, 21 years old or younger, or undergoing active cancer treatment.

A "Cancer Signal Detected" result is not a diagnosis of cancer. The results of the Galleri test must be confirmed by diagnostic evaluation recommended by qualified healthcare professionals in accordance with standard medical practice. These results should be interpreted in the context of the individual's clinical risk factors. Diagnostic decisions are the responsibility of the treating physician.

A "Cancer Signal Not Detected" result does not eliminate the possibility that a cancer is present or will occur in the future. Individuals who receive a "Cancer Signal Not Detected" result should continue with all recommended cancer screening options at intervals appropriate for the individual. The use of the Galleri test should not replace, supersede, or otherwise alter the use or frequency of standard of care cancer screening or detection modalities.

The Galleri test may not detect a cancer signal in all cancers; cancers evaluated in the CCGA sub-study are listed at www.galleri.com/test-report. The test performance in cancer classes not observed in CCGA and PATHFINDER is unknown. If a cancer signal is detected, the Galleri test also reports one or two Cancer Signal Origins, which must be confirmed by diagnostic evaluation.

In some cases, the Galleri test may produce a "Cancer Signal Detected" result, but follow-up diagnostic evaluation may not result in a cancer diagnosis. This could mean that the individual has a cancer that is difficult to identify by the selected follow-up diagnostic evaluation, that the individual has cancer but it is located elsewhere, or that the individual does not have cancer and the Galleri test result is a false positive.

Sensitivity and Cancer Signal Origin accuracy observed in cancer participants from the case-control CCGA sub-study may be higher than in the general screening population because cancer signals may be stronger in cancers that are detected by standard medical practice. The positive predictive value reported in the PATHFINDER study may be underestimated because only participants with "Cancer Signal Detected" results from the earlier version of the Galleri test had a diagnostic evaluation to establish clinical cancer status.

Performance of sequential Galleri tests has not been evaluated.

GRAIL's clinical laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) and accredited by the College of American Pathologists (CAP). The Galleri test was developed, and its performance characteristics were determined by GRAIL. The Galleri test has not been cleared or approved by the Food and Drug Administration. GRAIL's clinical laboratory is regulated under CLIA to perform high-complexity testing. The Galleri test is intended for clinical purposes.

The Galleri test can be ordered by a licensed practitioner only.

References

1. Liu MC, Oxnard GR, Klein EA, et al. CCGA Consortium. Sensitive and specific multi-cancer detection and localization using methylation signatures in cell-free DNA. *Ann Oncol.* 2020;31(6):745-759.
2. Klein EA, Richards D, Cohn A, et al. Clinical validation of a targeted methylation-based multi-cancer early detection test using an independent validation set. *Ann Oncol.* 2021;32(9):1167-1177.
3. Beer TM, McDonnell CH III, Nadauld L, et al. A prespecified interim analysis of the PATHFINDER study: Performance of a multi-cancer early detection test in support of clinical implementation. Presented at: 2021 American Society of Clinical Oncology Annual Meeting; June 4-8, 2021. Abstract 3070.
4. Beer TM, McDonnell CH III, Nadauld L, et al. Interim results of PATHFINDER, a clinical use study using a methylation-based multi-cancer early detection test. Presented at: 2021 American Society of Clinical Oncology Annual Meeting; June 4-8, 2021. Abstract 3010.